

Lip Filler Anatomy & Techniques

Injection mapping, product selection, danger zones, and clinical pearls

LIP ANATOMY – LAYERS & LANDMARKS

Structure	Location	Clinical Relevance
White roll	Pale line at cutaneous-vermilion junction	Preserve or enhance with border filler — loss looks unnatural
Vermilion border	Junction between skin and lip mucosa	Primary target for lip definition filler — intradermal threading
Dry mucosa	Visible pink lip — modified mucosa	Submucosal plane for volume — serial puncture technique
Wet mucosa	Inner lip surface — always moist	Deep submucosal placement — subtle body enhancement
Philtrum columns	Vertical ridges between Cupid's bow and nose	Light filler ok — avoid deep midline — superior labial artery runs here
Cupid's bow	M-shaped upper lip border peaks	Most impactful aesthetic result with least volume
Superior labial artery	Runs within upper lip substance	Variable depth — aspirate with needle, cannula preferred
Inferior labial artery	Runs within lower lip substance	Same precautions as superior labial artery

INJECTION TECHNIQUES

Technique	Best For	Notes
Linear threading anterograde	Vermilion border definition	Follow border exactly — maintain white roll
Serial puncture	Lip body volume	0.02–0.05mL per deposit — small aliquots
Fanning (cannula)	Overall volume, minimal bruising	Single entry lateral, fan across body
Bolus	Philtrum column enhancement	Tiny bolus only — 0.01–0.02mL per column
Cross-hatching	Thin lips needing structural support	Requires advanced technique

PRODUCT SELECTION BY GOAL

Goal	Product Choice	G-Prime	Volume Range
Subtle definition only	Volbella / Belotero Balance	Low	0.3–0.5mL total
Natural volume + definition	Juvederm Ultra / Kysse	Low-Med	0.5–1mL total
Significant volume add	Juvederm Ultra Plus	Medium	1–1.5mL total
Border only / lip flip support	Volbella / Restylane Silk	Very low	0.1–0.3mL
Restore age-related volume loss	Restylane Kysse / Defyne	Low-Med	0.5–1mL total

DANGER ZONES – LIPS

- ⚠️ Superior labial artery: runs variably within lip body — position changes between individuals. Aspirate before each deposit when using needles.
- ⚠️ Avoid deep injection at midline philtrum — superior labial artery runs here in most patients.
- ⚠️ Oral commissure / modiolus: multiple muscle convergence point — inject conservatively, avoid deep bolus.
- ⚠️ Blanching during injection = STOP immediately. Massage vigorously. Have hyaluronidase ready in treatment room at all times.

LIP FLIP VS LIP FILLER – WHEN TO USE WHICH		CLINICAL PEARLS	
LIP FLIP (BOTOX 4–6 UNITS)	LIP FILLER (HA 0.5–1.5ML)		
No volume added — eversion only	Adds actual volume and definition	14-day rule	Always assess at 14 days — do not add more filler before swelling fully resolves
Shows more of the lip you already have	Can reshape, define, and volumize	Pre-treatment	No blood thinners, alcohol, or NSAIDs 48–72 hrs before. Manage HSV with prophylactic antivirals if history present
Lasts 6–8 weeks	Lasts 6–12+ months		
Cannot replace lost volume	Reversible with hyaluronidase	Cannula entry	Entry point lateral to oral commissure using needle, then switch to 25–27g blunt cannula for body
Best: gummy smile + subtle enhancement	Best: volume loss, thin lips, definition		
Cost: \$50–80 typically	Cost: \$500–900 typically	Natural movement	Test lip movement throughout injection — patient should still be able to purse, smile, and speak naturally
Onset: 3–5 days	Swelling: peaks 24–72 hrs, resolves 2 wks		
		Cold compress	Apply immediately post-treatment to reduce swelling. Arnica gel helps bruising. Avoid heat 24 hrs